

General information

Startup name

ClinicalHours

Founders' names and contact information:

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University Affiliation:

Both founders are at Texas A&M University.

Website and social media links:

clinicalhours.org

IG: clinicalhours

Introduction video:

 IMG_2801.MOV

Company and product details

Company overview: ClinicalHours connects pre-med students with clinical volunteering opportunities at hospitals and clinics, helping them build the hours and experience they need for medical school.

The problem: What is the specific problem you are solving, and for whom?

Pre-med students have no centralized place to find clinical volunteer opportunities. They cold-call hospitals, send mass emails, maintain spreadsheets, and sometimes even walk in just to reach out and track who they've contacted. On the other hand, hospitals are flooded with disorganized student inquiries, especially during peak seasons, and lack a system for managing applications, all of which increases administrative workload and costs. Neither side has infrastructure. Both sides waste hours on a process that should take minutes.

The solution: How does your product or service solve this problem?

ClinicalHours is a two-sided marketplace where hospitals post opportunities and manage applicants from a single dashboard, and students find, apply for, and track their clinical hours in one place. We remove excessive email chains and long spreadsheets from the process for both sides. Students browse opportunities and apply directly. Hospitals get an organized pipeline with customizable applications, applicant filtering, and schedule management, replacing the inbox chaos entirely.

Beyond just finding and applying to opportunities, students also have access to an experience tracker where they can log their volunteer hours and reflections, providing an all-in-one portal with over 6 AI tools to help them apply to med school.

Stage of development

ClinicalHours is fully launched at clinicalhours.org.

Milestones:

We have traction and competition milestones.

Traction:

- Landed first B2B pilot partner: BCS Free Health Clinic
 - Launching waitlist for fall semester applications soon for this clinic specifically
- Reached 200+ users with a predicted increase of 50+ with BCS onboarding.
- In talks with DFW Clinic to join pilot (potentially paid partnership)

Competition:

- Good Bull Pitch: 3rd place, \$200
- Ideas Challenge Finalists
- Meloy Kickstart Launch Accelerator (3/11 teams)

Traction metrics:

Supply Side:

- 1 clinical partnership

Demand Side:

- 200 registered students

We've spent the last month building our application portals and speaking to hospitals, clinics, and professionals. We are opening a waitlist for interested students to volunteer with our partner clinic soon, to gauge interest and serve as the initial step for our pilot. We also expect a significant increase in users when we onboard BCS Free Health Clinic's current volunteers to the site.

Unique value proposition:

ClinicalHours is the only platform built specifically for clinical volunteering; it's not a job board or a general volunteering site. We own both sides of the marketplace in this niche. We aren't competing with existing platforms like VolunteerMatch; we're filling a gap for students seeking volunteer opportunities. For students, no other platform offers a centralized place to discover opportunities, apply, track hours, and log reflections. For hospitals, no other tool is built specifically to manage pre-med volunteers. Existing solutions, such as Volgistics, are either generic volunteer management software or manual processes. Our AI integration further sets us apart from our competitors. Opportunity matching means students find better fits faster. Application sorting means hospitals spend less time sifting through unqualified candidates. Neither side gets this anywhere else, and we provide both simultaneously.

Beyond the core hospital-student marketplace and benefits, we also provide our students with various tools, specific to med school applications, that incentivize them to use us throughout the process, from volunteer applications, tracking hours, and logging experiences, all the way through applying for med school. These tools are all AI-driven, use logged experiences to help students make the best possible application, and include an AMCAS generator, a competency map, and a secondary essay helper. We revolutionize this market by providing everything a student needs to curate a successful medical school application, all in one place.

The Market:

Target market:

Our primary customers are pre-med students and hospitals/clinics with structured volunteer programs. However, our TAM increases as our product evolves. For example, our most recent change was adding a volunteer tracker to aid hospitals in the post-application process of managing their volunteers. This feature gets us closer to competing with systems like Vsys and Volgistics, which is one of our goals on the hospital side.

Our current emphasis on smaller clinics also puts us in the unique position to introduce structured volunteer programs to clinics that don't currently have one, further expanding our total market and potential customer base.

Beyond this, our future scope is enterprise-focused: scaling to university pre-med advising offices. We want to sell our services directly to them, allowing them to manage their students, track progress, and handle meetings all through our platform.

Market size:

Assumptions for the student side: Conservative premium rate and conversion, driven by testing and market research to identify the ideal price point. All numbers are based on the CURRENT rate of \$5/mo. The # of serious pre-med students grows each year.

Assumptions for the hospital/clinic side: Assume we ONLY work with programs that already take volunteers (our services could eventually convince clinics to start; we can create demand).

TAM is based on a nationwide basis and assumes we are superior to all competition. SAM is nationwide, but makes rough estimates to account for competition and focuses on smaller clinics. SOM is a one-year, Texas-focused outlook that is also limited by the feasibility of land partnerships. It is more of a revenue outlook than a market projection. The student-side projection narrows based on a University's reachability across all three markets.

TAM (Student Side, Nationwide)

- 54,699 students apply annually (AAMC 2025, accelerating rate)
- Average 3.5 years yields 191,000 serious pre-med applicants in the pipeline nationally
- Convert 5% to premium at \$60/yr = **\$573,000**

TAM (Hospital/Clinic, Nationwide)

- 6,100 Hospitals, 17,000 Federally Qualified Health Centers (FQHCs), 1,400 Free Clinics, and 14,000 Urgent Care Centers.
- Only 62% of urgent cares are not large chains (Journal of Urgent Care Medicine), yielding a total of 33,180 locations
- This would yield a total market of 33,180 locations* \$750/location = **\$24.9 million**

SAM (Student Side, Nationwide)

- 54,699 students apply annually (AAMC 2025, accelerating rate)
- Average 3.5 years yields 191,000 serious pre-med applicants in the pipeline nationally
- 950 universities produce med-school applicants nationally (AAMC)
- Assume 3 reachable pre-med orgs at each school (more successful + pre-med focused schools) = 2,850 orgs.
- Assume an average org size of 100 ACTIVE members
- Reach 25% of students (campus ambassador program) = 71,250 reachable students
- Convert 5% to premium at \$60/yr = **\$213,750**

SAM (Hospital/Clinic, Nationwide)

- 17,000 Federally Qualified Health Center (FQHC), 1,400 Free Clinics, and 8,680 unchained Urgent Care Centers
- Conservative Assumption that 25% of unchained Urgent Care takes volunteers, yielding 17,770 locations (NACHC and NAFC source)
- Then, we make the assumption that 30% have no structured, or inferior, volunteer management systems, which gives 5,331 locations
- This yields a SAM of **\$4 million**

SOM (Student side, 1-6 month outlook, Texas focused)

- 7,220 Texas med school applicants, 75% Texas residents = 5,415 Texas resident applicants a year
- Average time spent searching for 3.5 yrs yields an average of 18,900 serious applicants at any given time
- First year outlook reaches 33% of pre-med organizations by size (via a campus ambassador program)
- We then reach 25% of the students through these orgs.
- Out of these students, 5% can be reasonably reached and converted to premium, yielding **\$ 4,750** in the first half-year.

SOM (Student side, 6-12 month outlook, Texas focused)

- Same assumptions, but the second year focuses on accelerating university partnerships, reaching 80% of pre-med organizations by size through word-of-mouth and expansion of the campus ambassador program.
- Same assumptions and calculations as the first half-year outlook yields **\$6,840**, for a **total of \$11,340**.

SOM (Hospital/Clinic side, 1-year outlook, Texas-focused)

- 71 Federally Qualified Health Centers (FQHC) and an estimated 550 free clinics (The Free Clinic Directory) for a total of 621.
- Assumption that 100% have active volunteer programs for pre-meds (due to the structure of free clinics, relying on volunteers).
- Assumption that 60% are willing to pay; we don't know yet exactly the perfect price point to maximize the ceiling on our pricing.
- \$750/yr is under \$1,000, so we assume this makes the decision simpler for larger clinics. However, it's a high cost for smaller clinics, which we account for in the 40%. This puts us at 372 clinics.
- In one year, we can realistically close 15% of this at \$750/yr/clinic, giving us **\$41,900**.

Competitive landscape:

Our closest competitors are general volunteer platforms such as VolunteerMatch and Idealist, backend volunteer management tools such as Volgistics, and Handshake. None of these platforms and tools solves the problem ClinicalHours solves. VolunteerMatch and Idealist are broad marketplaces with no specific focus on clinical volunteering. Volgistics is a backend management tool; it has no student-facing discovery and is overkill for smaller clinics that just need a simple application pipeline. Our closest *potential* competitor is Handshake, and we win in our niche due to our speed and vast network effect. Handshake grew by partnering with Universities nationwide, making them irreplaceable in their field. Similarly, we will grow by partnering with hospitals and clinics nationwide.

We sit in the gap: a two-sided marketplace for clinical volunteering. Our AI features (opportunity matching, application sorting, student app review, etc.) add a layer of intelligence to the volunteer application process that no competitor in this niche offers. Our real advantage is the network effect: every hospital we add attracts more students, and every student we add makes the platform more valuable to hospitals. Once established, a competitor would have to rebuild both sides of the network from scratch to displace us.

The Team:

Background and expertise:

We are uniquely qualified to build ClinicalHours because we represent both sides of the market we're serving. Raghav is a pre-med student who lived this problem firsthand, tracking hospital outreach on spreadsheets before we built the platform. The idea came from Raghav literally trying to solve his own problem.

He brings direct reliability with students, truly understanding their problems. Shivam is an Industrial and Systems Engineering student with hands-on experience in app development, product thinking, and business research and launch. Together, we've taken ClinicalHours from an idea in December to a live product with 200+ organic users in under three months.

Key roles:

Raghav has primarily focused on developing new features and core product capabilities, leveraging his experience and connections with pre-med students to enhance ClinicalHours' functionality.

Shivam focuses on go-to-market strategy and long-term product direction, such as bringing ClinicalHours to the public through targeted outreach, pitch competitions, campus ambassador programs, and hospital partnerships.

Advisors and mentors:

We have been lucky to meet incredible advisors over the past semester.

William Shaw (Volunteer Manager at St. Joseph Health)

- Validated our idea; gave us insights on competition (Volgistics) that led to a critical pivot from hospital-focused to student-focused.

Mark Radulovich (Founder of Prota Financial, mentor)

- Met at Good Bull Pitch, advises us on pitching, market strategy, and financial insights.

Rajeev Prashar (Enterprise Architecture & Strategy Leader at Ascension)

- Ascension owns 100 hospitals and over 2,000 clinics.
- Validated what William Shaw told us: Ascension-owned hospitals have volunteer management systems in place.
- Told us smaller clinics do not, further validating our current business focus and initial pivot.
- Introduced an exit plan strategy: partner with Athenahealth and use our data collection to sell our software to university pre-med advising offices.

Financials and Fundraising

Investment ask:

We're looking for \$10,000 in non-dilutive funding. Use of funds is as follows:

Legal — \$2,500

- Texas LLC formation (state filing fee): ~\$300
- Clerky or equivalent for operating agreement + founder docs: ~\$500
- Termly or equivalent for privacy policy + terms of service: ~\$200
- Contract templates for clinic B2B agreements: ~\$500
- Buffer for misc legal needs: ~\$1,000

Product — \$1,500

- Apple Developer Program (annual): \$99
- Supabase (free tier, no cost projected): \$0
- OpenAI / Anthropic API usage (AI features): ~\$600
- Vercel Pro (hosting, if free tier exceeded): ~\$200
- Lovable + Cursor subscriptions (~\$40/mo x 6 months): ~\$600

Growth — \$4,800

- Pre-med influencer content (\$25/video x ~20 videos): \$500
- Campus outreach materials (flyers, tabling): \$300
- Event costs (pitch competitions, travel, registration): \$700
- Ambassador program (commission-based, no upfront cost): \$0
- Paid social/targeted ads to pre-med students: \$3,300

Operations — \$1,200

- Domain + hosting misc: \$100
- Claude Code + productivity tools (~\$40/mo x 12 months): \$480
- Miscellaneous (printing, shipping, unforeseen): \$620

Valuation:

If we were raising a pre-seed round today, we estimate a pre-money valuation of approximately \$2.7M, based on two independent methods applied at our current stage.

The Berkus Method is designed specifically for pre-revenue startups and uses five risk factors, each worth \$1,000,000:

- The soundness of the idea (7/10). The problem is real and well-documented, but the difficulty of implementation on both sides simultaneously is also very real.
- A working prototype (6/10). Our app is completely live and being used by a real clinic. However, our student-side features are not driving growth, and the problem is that the AI premium features are not working well.
- Team quality (6/10). We are two freshman founders, one pre-med, one engineering. The founder-market fit is absolutely there; we represent both sides of the market. We went from idea to launch in two months. Our age and experience are a strength and a weakness. We have time to build and learn throughout college. However, we don't have prior exits or deep industry experience to leverage.
- Strategic relationships (6/10). We have a partner clinic and connections with a volunteer manager at a large hospital chain (St. Joseph), a strategy leader at Ascension, and a financial manager. While all of these relationships have been beneficial and will continue to help us, there is much more we need to do to become truly successful in this field. More clinical partners, more mentors across fields, such as marketing on the student side.
- Early market feedback (2/10): While we have over 200 users and a clinical partnership, much of the market is unexplored. Over the next few months, we plan to expand deep into the clinic space and learn more about our target consumer, shaping our market predictions as well.

Applied to ClinicalHours, this method yields a valuation of \$2.7M, driven primarily by our opinion of our ability to identify and address the problem and our prototype's ability to solve it.

We can also compare ClinicalHours to Handshake as the closest product that supports the same range of features and usability. Handshake raised its seed round in 2015 with a live product and early university partnerships at an estimated \$4-8M. ClinicalHours operates in a significantly narrower, mostly uncontested niche. We also have a smaller founding team and fewer prior industry connections than Handshake's founders. I believe these factors validate our Berkus forecast, given that ClinicalHours would be valued at 50-70% less than Handshake at the same stage. **ClinicalHours is comparable to Handshake at this stage because of the product and competitive position, making this forecast viable.**

Existing funding:

We received \$200 in prize funding from the Good Bull Pitch competition (3rd place). No outside capital, angel investment, or institutional funding has been raised to date.

Financial projections:

Year 1 (Texas pilot)

- 5 clinic partners at \$750/year = \$3,750
- 190 student premium subscribers at \$60/year = \$11,400
- Total: ~\$15,150

Note: Year 1 revenue is conservative. First half focused on product and partnership infrastructure, second half on close rate.

Year 2 (Texas scale)

- 25 clinic partners at \$750/year = \$18,750
 - 600 student premium subscribers at \$60/year = \$36,000
 - Total: ~\$54,750
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Year 3 (Regional expansion — TX + neighboring states)

- 80 clinic partners at \$875/year avg = \$70,000
 - 1,800 student premium subscribers at \$60/year = \$108,000
 - Total: ~\$178,000
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Year 4 (National + advisor dashboard launch)

- 200 clinic partners at \$1,000/year = \$200,000
 - Advisor dashboard: 15 university pre-med offices at \$5,000/year = \$75,000
 - 4,500 student premium subscribers at \$60/year = \$270,000
 - Total: ~\$545,000
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Year 5 (National scale)

- 450 clinic partners at \$1,000/year = \$450,000
- Advisor dashboard: 50 university offices at \$5,000/year = \$250,000
- 9,000 student premium subscribers at \$60/year = \$540,000
- Total: ~\$1,240,000

Assumptions

Clinic side:

- Addressable Texas market: 621 free/community clinics
- Close rate grows from 1% (Y1) to 4% (Y2) to national expansion Y3+
- Price increases from \$750 to \$1,000 as product matures
- Churn is assumed at 10% annually once partnerships stabilize

Student side:

- Premium conversion held at 5% of reachable users throughout
- Reachable users grow via the ambassador program, scaling from 2-5 schools (Y1) to 30+ (Y3) to 100+ (Y5)
- Annual churn is naturally high (~60%), given one application cycle per student, offset by new cohort enrollment each year

Advisor dashboard (Y4+):

- Targets university pre-med advising offices, not individual students
- \$5,000/year per office is below comparable advising SaaS tools (\$8,000-15,000)
- Conversion assumes warm intros through existing student ambassador relationships
- This becomes the highest-margin, lowest-churn revenue line

General:

- No paid marketing assumed until Y3
- Team stays lean through Y2, first hire assumed Y3
- Figures are gross revenue, not accounting for hosting/API costs, which scale modestly with usage

Attachments:

 **ClinicalHoursBusinessPlan.pdf**

 **ClinicalHours_Financial_Model_v3 (2).xlsx**

 **Raghavsai Tirupati.pdf**

 **ShivamKanodiaResume (19).pdf**